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UNITED STATES DISTRICT COURT

NORTHERN DISTRICT OF CALIFORNIA

SAN FRANCISCO DIVISION

BRENDA HORSLEY, *et al.*, *Plaintiffs*, vs. KAISER FOUNDATION HOSPITALS, INC., GREG ADAMS, ANDREW BINDMAN, M.D., TOMAS ARAGON, AND GOVERNOR GAVIN NEWSOM, *Defendants*.	Case No.: 3:23-CV-5628 AMO **PLAINTIFFS' MEMORANDUM RE CLEARLY ESTABLISHED RIGHTS**

I. Introduction

This memorandum addresses the Court's request regarding whether Plaintiffs' Third Amended Complaint is futile, specifically focusing on whether Plaintiffs have alleged a clearly established right that State Defendants violated. State Defendants complain that this is the third time Plaintiffs wish to amend their Complaint, but State Defendants fail remind the court that the first and second amendments were solely to add one plaintiff to the litigation each time. Thus, Plaintiffs' proposed Third Amended Complaint would be the first substantive amendment to Plaintiffs' Complaint.

II. Qualified Immunity and Clearly Established Right

"The doctrine of qualified immunity protects government officials 'from liability for civil damages insofar as their conduct does not violate clearly established statutory or constitutional rights of which a reasonable person would have known.'" *Pearson v. Callahan*, 555 U.S. 223 (2009). As the Supreme Court explained,

> In the civil sphere, we have explained that qualified immunity seeks to ensure that defendants 'reasonably can anticipate when their conduct may give rise to liability…by attaching liability only if "[t]he contours of the right [violated are] sufficiently clear that a reasonable official would understand that what he is doing violates that right.[citation omitted] So conceived, the object of the "clearly established" immunity standard is not different from that of "fair warning" as it relates to law "made specific" for the purpose of validly applying [the criminal equivalent of § 1983]."[1]

The *Lanier* Court added,

> But general statements of the law are not inherently incapable of giving fair and clear warning, and in other instances a general constitutional rule already identified in the decisional law may apply with obvious clarity to the specific conduct in question, even though 'the very action in question has [not] previously been held unlawful. [citation omitted] As Judge Daughtrey noted in her dissenting opinion in this case, "[t]he easiest cases don't even arise. There has never been . . . a section 1983 case accusing welfare officials of selling foster children into slavery; it does not follow that if such a case arose, the officials would be immune from damages [or criminal] liability."[2]

[1] *US v. Lanier*, 520 U.S. 259 (1997) citing *Anderson v. Creighton*, 483 U.S. 635, 640 (1987)

[2] *Id.*

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The Ninth Circuit explained the two-part analysis for determining whether qualified immunity is appropriate in a § 1983 case:

> First, examine whether a constitutional right has been violated on the facts alleged, and
>
> Second, if there was a violation, examine whether the Constitutional rights were clearly established.
>
> As to the second inquiry, the Supreme Court has held that "[i]f the law did not put the officer on notice that his conduct would be clearly unlawful, summary judgment based on qualified immunity is appropriate."[3]

In the case at bar, the constitutional rights violated are the constitutional rights of liberty (bodily integrity and autonomy), property (State-issued healthcare licenses and statutory entitlements), due process (substantive and procedural), and equal protection, all of which were violated when Defendants issued a Health Order requiring Plaintiffs to inject federally owned investigational drugs into their bodies as a condition of working within the State's licensed healthcare industry. The Fourteenth Amendment rights owed to Plaintiffs by Defendants could not be deprived before notice was given and a meaningful opportunity to air their protest were provided. Rather, State Defendants unilaterally terminated Plaintiffs' chosen careers solely because Plaintiffs exercised their fundamental right to refuse the federally owned INDs, which State Defendants assured the federal government they would not do. This conduct by State Defendants shocks the conscience.

In addition to examining the constitutional rights, the Ninth Circuit examines the challenged conduct to determine whether it is discretionary or ministerial. "[M]inisterial acts are unshielded by qualified immunity, which protects 'only actions taken pursuant to discretionary functions'" *Groten v. California*, 251 F.3d 844 (9th Cir. 2001) In *Groten*, clerks in the California Office of Real Estate Appraisers refused to give Mr. Groten an application for a temporary real estate license. The clerks were

[3] *Boyd v. Benton County*, 374 F.3d 773 (9th Cir. 2004)

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denied qualified immunity because the act of providing an application requires no discretion and was thus ministerial, leading to the Court to deny the defendants' 12(b)(6) Motion to Dismiss.

The Ninth Circuit reiterated its position in *Rieman v. Vazquez*, 96 F.4th 1085 (9th Cir. 2024) holding that two social workers were not entitled to absolute or qualified immunity because the challenged conduct only involved the ministerial function of providing notice of a hearing, which the defendants breached. *See also, Lacey v. Maricopa County*, 693 F.3d 896 (9th Cir. 2012) (denying absolute immunity for conduct taken outside the prosecutor's scope of authority); *Frudden v. Pilling*, 877 F.3d 821 (9th Cir. 2017)(denying qualified immunity because defendants had discretionary authority to issue a policy but lacked authority to apply the policy to the deprivation of the plaintiffs' Fourteenth Amendment rights.)

Therefore, if the challenged conduct falls outside the State Defendants' discretionary authority or the challenged conduct only involves a ministerial duty, then they are "unshielded" from qualified immunity. Plaintiffs have alleged facts that State Defendants acted outside the scope of their authority (by issuing their Health Order relying upon investigational drugs for compliance) and that they failed to exercise their ministerial duty to obtain Plaintiffs' legally effective informed consent (i.e., simply accepting Plaintiffs' decision to refuse).

III. The Gravamen of the Case

The Investigational New Drugs (INDs) in question were, at all relevant times, the property of the United States Government (USG) (Doc. 33, ¶ 172). Constitutional principles and applicable case law prohibit the USG from requiring individuals be injected with investigational drugs as a condition of receiving any governmental benefit. The USG delegated its authority to administer these drugs to the State of California, along with the accompanying constitutional restrictions and statutory obligations under its CDC Program and Provider Agreement. (Doc 33. ¶¶ 60, 86, 187, 201, 233, 274, 293) California, in turn, delegated this authority and these duties and obligations to Kaiser (Doc 33. ¶¶, 305, 306). Consequently, the constitutional protections against mandatory medical treatment by the government extended to all

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levels of this delegated authority, from the federal government through state agencies and normally private entities like Kaiser, who willfully agreed to perform on behalf of the State.

Investigational drugs do not have a legal indication to treat any known medical issue and therefore are legally considered investigational treatments providing Plaintiffs with the unqualified and clearly established right to refuse without pressure or penalty. (Doc 33. ¶¶ 191,192,193) State Defendants did not dispute these allegations in their Motion to Dismiss.

IV. State Defendants Ministerial Duties

a. CDC COVID-19 Vaccination Program

Under the CDC Program, California had the following *ministerial* duties:

1. Receive, store, distribute, and administer COVID-19 drugs (Doc 33, ¶199);
2. Accept the Plaintiffs' chosen option of whether to accept or refuse;
3. Comply with "FDA's requirements, including EUA-related requirements" *Id.*;
4. Ensure Kaiser signed the Provider Agreement and agreed to the conditions under the CDC Program *Id.*;
5. "[C]losely monitor activities at the local level to ensure the COVID-19 Vaccination Program is implemented throughout the jurisdiction in adherence with federal guidance and requirements" (Doc. 40, p. 16);
6. "[H]elp the public to understand the key differences in FDA emergency use authorization and FDA approval" *Id.*;
7. Ensure Kaiser complied with the requirements of the Provider Agreement and any issued EUA. (Doc 33, FN 89; Doc. 40, pp. 4, 17, 18, 20);
8. Instruct Kaiser how to obtain and administer the drugs under the EUA (Doc. 40, p. 17);
9. Instruct Kaiser to provide to every potential recipient a Fact Sheet, which serves as the drug's label and part of informed consent. *Id.*

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10. Inform the public of the "key differences" between EUA and licensed drugs;

11. Inform the healthcare industry of the right to refuse EUA INDs without pressure or penalty.

Under the CDC Program, California had the following *discretionary* duties:

1. Recruit private parties to administer the federally owned drugs by deciding which entities to recruit, how many were needed, where the drugs would be available across the state, and when the drugs would be offered.

State Defendants did not have discretionary authority under the CDC Program, EUAs, or the State's FWA to make injection of INDs a condition of Plaintiffs working in the State's licensed healthcare industry, so the Health Order was issued outside the scope of their authority. State Defendants cannot be afforded qualified immunity for willfully violating ministerial duties they volunteered to perform. In fact, the Health Order, as applied, violated every one of State Defendants' ministerial duties.

b. Federal Wide Assurance Agreement

The State's FWA00000681 with the USG requires the State to comply with the regulatory framework under 45 CFR § 46.116 and the Belmont Report whenever the State offers humans federally funded investigational drugs such as those under the CDC Program. (Doc 33. ¶ 281) The FWA requires State Defendants to perform the following ministerial duties:

1. Ensure Plaintiffs were not under "coercion," "undue influence," or "unjustifiable pressures" to be injected with the drugs;

2. Ensure Plaintiffs were not penalized for exercising their right to refuse.

3. State Defendants refused to perform these ministerial duties and are thus unshielded by qualified immunity.

c. 21 U.S.C. §360bbb-3

As the "emergency response stakeholder" under each COVID-19 EUA, State Defendants have the following ministerial duties:

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1. Ensure that its "vaccination providers" (i.e., Kaiser) comply with the EUA and the Provider Agreement.

2. Ensure Kaiser performed the ministerial duty of accepting Plaintiffs' option to accept or refuse without pressure or penalty.

State Defendants do not have discretionary authority to amend the "conditions of authorization" under each EUA, nor establish a prohibited act (refusing) under 21 U.S.C. § 331, nor assign a penalty under 21 U.S.C. § 333 for refusing, nor void the option to refuse under 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) which their Health Order effectuated, and thus are not entitled to qualified immunity.

V. Clearly Established Rights

Plaintiffs' clearly established right to refuse investigational drugs is longstanding, as are their rights of liberty, property, due process, and equal protection. Twenty years ago, Judge Emmett Sullivan of the District of Washington, D.C., spoke on the importance of not mandating investigational drugs when he issued an injunction against the federal government mandating civilian and service members to inject an investigational anthrax vaccine into their bodies under threat of penalty, stating,

> The Court is persuaded that the right to bodily integrity and the importance of complying with legal requirements, even in the face of requirements that may potentially be inconvenient or burdensome, are among the highest public policy concerns one could articulate. *Doe v. Rumsfeld*, 341 F. Supp. 2d 1 (D.D.C. 2004).

The *Rumsfeld* decision demonstrates that the right to refuse unwanted investigational drugs was clearly established as of 2004. Additionally, the right to refuse without penalty was clearly established when the EUA letter authorizing the emergency use of the investigational anthrax vaccine discussed in *Rumsfeld* was published in the Federal Register EUA (Doc 33. ¶¶ 130-133).

State Defendants cannot constitutionally condition Plaintiffs' right to work within the State's licensed healthcare industry or condition Plaintiffs' use of their own property (State-issued healthcare licenses) upon submitting to unwanted investigational drugs or treatments offered under an EUA, PREP Act, or the CDC COVID-19 Vaccination Program and such a right to refuse the INDs without pressure or penalty was clearly established at the time Defendants issued their Health Order.

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State Defendants claim that "Plaintiffs cannot establish that a requirement that health care workers be vaccinated with EUA vaccines in the face of a deadly pandemic violated 'clearly established' rights or law." (Doc. 55, p. 7 at 20-22). State Defendants' attempt to create a "deadly pandemic" exception to Plaintiffs' constitutional rights ignores Plaintiffs' quote from the U.S. Supreme Court on p. 2 of Plaintiffs' Complaint: **"even in a pandemic, the Constitution cannot be put away and forgotten."** *Roman Catholic Diocese of Brooklyn v. Cuomo*, 141 S.Ct. 63, 208, L.Ed.2d 206 (2020). (Doc 33, p. 2)

Furthermore, in *Perry v. Sindermann*, 408 U.S. 593, 92 S.Ct. 2694, 33 L.Ed.2d 570 (1972), the Supreme Court established that the government cannot condition the receipt of a governmental benefit on surrendering a constitutional right, even if the individual has no entitlement to that benefit. As stated in *Perry*:

> "For at least a quarter-century, this Court has made clear that even though a person has no 'right' to a valuable governmental benefit and even though the government may deny him the benefit for any number of reasons, there are some reasons upon which the government may not rely. It may not deny a benefit to a person on a basis that infringes his constitutionally protected interests." *Perry v. Sindermann*, 408 U.S. 593, 92 S.Ct. 2694, 33 L.Ed.2d 570 (1972)

In the case at bar, however, Plaintiffs do have a right to a valuable governmental benefit – using their State-issued healthcare licenses or generally working in the State-licensed healthcare industry – and to the constitutional and statutory entitlement right to refuse unwanted investigational drugs or medical treatment under the CDC Program and the State's FWA. The Ninth Circuit was unambiguous stating, "This court similarly has observed that 'it is well-recognized that the pursuit of an occupation or profession is a protected liberty interest that extends across a broad range of lawful occupations'" *Dittman v. California*, 191 F.3d 1020 (9th Cir. 1999). Moreover, the Surpeme Court has recognized that a state-issued license is a property right for the purpose of due process under *Goldberg v. Kelly*, 397 U.S. 254 (1970).

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1. Investigational New Drugs are Not Vaccines

Plaintiffs allege that the only products made available to them for compliance with the Health Order were classified by the FDA as investigational new drugs (INDs), not as FDA-licensed vaccines. This distinction is crucial, as it implicates fundamental rights and statutory protections, which rights are the gravamen of the case. State Defendants call the drugs "vaccines," which black letter law dictates they cannot be. A vaccine is a drug or biologic that has been granted a biologics license application pursuant to 21 U.S.C. § 355(a) with a legal indication to inoculate a person from a specific disease. By contrast, an IND does not have a legal indication for its safety and efficacy to treat, cure, or prevent any known disease, and an IND's use is legally considered investigational. COMIRNATY® is an FDA-licensed vaccine, and the "Pfizer-BioNTech COVID-19 Vaccine" (uppercase V) is an FDA-classified investigational new drug. Plaintiffs clearly alleged that COMIRNATY® was never made available to the public, which claim was supported by judicially noticeable documents.[4] State Defendants refuse to address the FDA's official notice[5] that it classified the drugs as investigational and provided nothing to this Court that credibly disputes the authority of the FDA's classification of the drugs. State Defendants have no authority to amend a drug's classification.

2. Right to Refuse Unwanted Investigational Medical Treatment

State Defendants cannot constitutionally mandate that Plaintiffs receive an unwanted medical treatment, and they cite to no case law providing them such authority. Such a condition would violate Plaintiffs' fundamental right to refuse unwanted medical treatment as discussed by the U.S. Supreme Court in *Cruzan v. Director, Missouri Dep't of Health*, 497 U.S. 261 (1990). There, the Court was forced to differentiate between the rights of competent and incompetent individuals. For competent individuals, like Plaintiffs herein, the Court reiterated the following clearly established right:

[4] Doc. 40, p. 12,13
[5] Doc 33-2, p. 9, Section G.

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> "On balance, the right to self-determination ordinarily outweighs any countervailing state interests, and competent persons generally are permitted to refuse medical treatment, even at the risk of death. Most of the cases that have held otherwise, unless they involved the interest in protecting innocent third parties, have concerned the patient's competency to make a rational and considered choice." *Cruzan* at 273, *citing In re Conroy*, 98 N.J. 321, 348, 486 A.2d 1209, 1223.

In reaching that decision regarding competent individuals, the Supreme Court noted that the right had been established as far back as 1891 when the Supreme Court held:

> "[n]o right is held more sacred or is more carefully guarded by the common law than the right of every individual to the possession and control of his own person, free from all restraint or interference of others unless by clear and unquestionable authority of law." *Union Pacific Railway Co. v. Botsford*, 141 U.S. 250 (1891) This notion of bodily integrity has been embodied in the requirement that informed consent is generally required for medical treatment. Justice Cardozo, while on the Court of Appeals of New York, aptly described this doctrine: "Every human being of adult years and sound mind has a right to determine what shall be done with his own body; and a surgeon who performs an operation without his patient's consent commits an assault, for which he is liable in damages." *Schloendorff v. Society of New York Hospital*, 211 N.Y. 125, 129- 130, 105 N.E. 92, 93 (1914).

Seven years after *Cruzan*, the U.S. Supreme Court reaffirmed its holding when it stated:

> "The right assumed in *Cruzan*, however, was not simply deduced from abstract concepts of personal autonomy. Given the common-law rule that forced medication was a battery, and the long legal tradition protecting the decision to refuse unwanted medical treatment, our assumption was entirely consistent with this Nation's history and constitutional traditions." *Washington v. Glucksberg*, 521 U.S. 702 (1997).

That same year, when confronted with an assisted suicide case, the Supreme Court again reiterated:

> In New York, as in most States, it is a crime to aid another to commit or attempt suicide, but patients may refuse even lifesaving medical treatment. (Footnotes omitted)
>
> * * *
>
> Everyone, regardless of physical condition, is entitled, if competent, to refuse unwanted lifesaving medical treatment; no one is permitted to assist a suicide. *Vacco v. Quill*, 521 U.S. 793 (1997).

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The Ninth Circuit also recently weighed in on the issue:

> In *Washington v. Glucksberg*, 521 U.S. 702 (1997), the Court explained that *Cruzan*'s posited "'right of a competent individual to refuse medical treatment'" was "entirely consistent with this Nation's history and constitutional traditions," in light of "the common-law rule that forced medication was a battery, and the long legal tradition protecting the decision to refuse unwanted medical treatment." *[Cruzan]* at 724–25 (citation omitted). Given these statements in *Glucksberg*, the right described there satisfies the history-based standards that the Court applies for recognizing "fundamental rights that are not mentioned anywhere in the Constitution." *Dobbs v. Jackson Women's Health Org.*, 597 U.S. 215, 237–38 (2022). The Supreme Court's caselaw thus clarifies that compulsory treatment for the health benefit of *the person treated*—as opposed to compulsory treatment for the health benefit of *others*—implicates the fundamental right to refuse medical treatment.

3. Right to Refuse EUA Investigational Medical Treatments.

In balancing the government's interests during a declared national emergency with individual rights, Congress unequivocally created statutory entitlements for individuals considering the use of EUA investigational drugs. The statutory framework created by Congress encompasses the right to be informed of the risks, benefits, and alternatives of the product, aligning with the principle of informed consent established in *Schloendorff v. Society of New York Hospital*, 211 N.Y. 125, 105 N.E. 92 (1914); the right to be notified of the option to accept or refuse the product, respecting the "significant liberty interest in avoiding the unwanted administration of medical treatment" recognized in *Cruzan, supra*; the entitlement to make autonomous health decisions, echoing the Supreme Court's affirmation of the importance of personal autonomy in medical decision-making, with which the Supremacy Clause preempted Defendants from interfering by issuing the Health Order as applied to the exclusive use of EUA INDs. [6]

[6] "[Congress's] intent to displace state law altogether can be inferred from a framework of regulation 'so pervasive . . . that Congress left no room for the States to supplement it' or where there is a 'federal interest. . . so dominant that the federal system will be assumed to preclude enforcement of state laws on the same subject.'" *Arizona v. United States*, 567 U.S. 387, 399 (2012) quoting *Rice v. Santa Fe Elevator Corp.*, 331 U.S. 218, 230 (1947)

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Notably, Congress declared that even in the face of chemical, biological, radiological, nuclear, or pandemic threats,[7] the government's legitimate interest includes safeguarding individual autonomy, which Congress, granting the HHS Secretary authority to introduce into commerce unlicensed drugs during a declared emergency simultaneously restricted that authority from requiring any person to participate, including receiving the product.[8] Furthermore, the Secretary's authority under the EUA Statute is nondelegable. By providing a federally secured right to refuse, Congress has created a "legitimate claim of entitlement" as defined in *Board of Regents of State Colls. v. Roth*, 408 U.S. 564, 576-77, 92 S.Ct. 2701, 33 L.Ed.2d 548 (1972), *overruled in part and on other grounds in Paul v. Davis*, 424 U.S. 693, 96 S.Ct. 1155, 47 L.Ed.2d 405 (1976), elevating the right to a protected property interest under the Due Process Clause. The statutory construct reflects a balance between the State issuing public Health Orders and individual liberty, resonating with the Supreme Court's consistent emphasis on protecting fundamental rights against government intrusion, as exemplified in *Griswold v. Connecticut*, 381 U.S. 479, 85 S.Ct. 1678, 14 L.Ed.2d 510 (1965), which, when applied to the instant action, protects the private choices of individuals to use investigational drugs or medical treatments offered under a federally funded program. The right to refuse unwanted investigational drug treatments is protected under the Fourteenth Amendment and is clearly established.

4. Right to Refuse a PREP Act Covered Countermeasure

The PREP Act establishes a liability immunity for certain covered persons "arising out of the administration to or the use by an individual of a covered countermeasure." This immunity vitiates an individual's fundamental right to pursue certain common law causes of action, including product liability, medical malpractice, fraud, and battery, as well as their right to seek tort remedies for bodily harm and compensation for financial and emotional damages arising out of the administration to or the use by an

[7] The U.S. Congress enacted "Project Bioshield" to respond to CBRN events and later included pandemics under the law. Public Law No: 108-276 (07/21/2004)

[8] 21 U.S.C. §360bbb-3(l)

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individual of those countermeasures. The loss of the Fourteenth Amendment due process right must only come from voluntary surrender, not through an *ultra vires* Health Order issued under threat of penalty, so Defendants penalizing Plaintiffs by completely depriving them of their right to work within the State's licensed healthcare industry gives rise to a § 1983 cause of action. The Supreme Court has ruled that if a government cannot constitutionally mandate a policy, then it cannot "produce a result which [it] could not command directly." *Perry v. Sindermann,* 408 U.S. 593, 92 S.Ct. 2694, 33 L.Ed.2d 570 (1972). Defendants violated Plaintiffs' clearly established rights to enjoy the public benefit of working in the State's licensed health care industry without the condition of injecting covered countermeasures into their bodies, thus State Defendants are not entitled to qualified immunity.

III. Conclusion

Plaintiffs' proposed Third Amended Complaint is not futile. In fact, it adds factual allegations that were recently held to be meritorious by the Ninth Circuit in *Health Freedom v. Carvalho*, 22-55908 (9th Cir. June 7, 2024). Moreover, Plaintiffs have plausibly alleged that their clearly established rights were violated by State Defendants. Additionally, under F.R.C.P. Rule 15, "[t]he court should freely give leave when justice so requires." Although named Third Amended Complaint, it would be the first substantive amendment to Plaintiffs' Complaint, brought after a groundbreaking decision in Health Freedom. Since significant constitutional and federal statutory rights are involved, Plaintiffs respectfully suggest that "justice so requires" the filing of their Third Amended Complaint.

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CERTIFICATE OF SERVICE

I hereby certify that on this 20th day of August, 2024, I presented the foregoing pleading to the Clerk of Court for filing and uploading to the CM/ECF system, which will send notification of such filing to all counsel of record.

/s/ David J. Schexnaydre
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